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Public Version

Steven Posnack

Principal Deputy Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology
Mary E. Switzer Building, 7033A, 330 C Street SW
Washington, DC 20201

Re: HHS Health Sector AI RFI (Docket ID.- HHS-ONC-2026-0001)

Dear Mr. Posnack,

BD (Becton, Dickinson and Company) applauds the ASTP for its leadership in accelerating the safe, effective use of AI in clinical care. We support the agency's goal of fostering public trust, reducing uncertainty that slows innovation, and aligning federal incentives to improve AI efficiency, lower costs and strengthen health outcomes.

BD is committed to ensuring patients have access to medical technologies, touching billions of patients and helping improve and save lives globally. In the United States, BD manufactures more medical devices than any other company, employing over 29,000 employees and operating more than 30 manufacturing sites and distribution centers. Throughout our 128-year history, BD has been a long-standing partner to the U.S. government in its efforts to ensure access to innovative medical technologies that are critical to American healthcare. Looking ahead, we are shaping the future of connected care through novel innovations in medication management, patient monitoring and pharmacy automation. By embedding AI and automation, we are streamlining complex clinical workflows and generating actionable insights that enable clinicians to deliver safer, more precise care for patients.

We believe that clear, predictable regulatory environment and modernized reimbursement pathways are essential to unlocking AI's full potential across the healthcare ecosystem. To achieve that goal, we wanted to offer the following recommendations for consideration:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The most significant barriers to private-sector innovation in AI stem from the absence of scalable and sustainable pathways to deploy high-value AI technologies in inpatient, procedural care and post-acute settings where healthcare costs and complexity are greatest. The agency has an opportunity to bend the cost curve by incentivizing the use of AI-enabled technologies in these settings to automate workflows, reduce redundancies, and support earlier clinical intervention. For example, AI-enabled hemodynamic monitoring during and after surgery can help improve patient safety and reduce complications such as acute kidney injury (AKI), myocardial injury, surgical site infections, and delirium. AI-enabled monitoring in obstetric care can reduce maternal hypotension and improve early identification of

postpartum hemorrhage. Such interventions can meaningfully improve outcomes while reducing length of stay and downstream costs.

Addressing these barriers will require modernized reimbursement models for AI embedded in medical devices, regulatory clarity, and established incentive structures that reward health systems for investing in deflationary AI technologies.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

From a regulatory perspective, FDA's risk-based framework provides a strong foundation for AI-enabled medical technologies, but more consistent implementation is needed to support adaptive systems. Encouragement of broader and more consistent use of FDA's Predetermined Change Control Plans (PCCPs) is needed, enabling responsible post-deployment updates without unnecessary resubmissions, and expand use of real-world evidence to support lifecycle oversight while maintaining patient safety. In addition, while the proposed HTI-5 rule makes meaningful progress in reducing certification burden and narrowing ONC's scope, it does not explicitly clarify how FDA-regulated medical devices that use EHR data are treated, leaving unresolved the risk of duplicative oversight for technologies already determine to be safe and effective by the FDA.

From a payment policy perspective, CMS should prioritize clearer and more predictable pathways for coverage, coding, and reimbursement of AI-enabled medical technologies, as payment uncertainty remains a primary barrier to adoption and investment in innovative AI products. Transitional coverage pathways, demonstration models, and targeted add-on payments could better align incentives with technologies that reduce burden, improve outcomes, or lower total cost of care. Given that many MedTech solutions involve capital intensive hardware and evolve through ongoing software enhancements, pilot programs should be designed with a clear mechanism to translate demonstrated value into durable coverage, coding, or payment pathways, enabling broader and more sustainable adoption over time. Specifically, we have the following recommendations with respect to payment policies:

Accounting for Devices that Streamline Operations while Improving Outcomes

We recommend that CMS consider alternative payment pathways for AI-enabled capital that can substantially reduce waste, enhance workflow efficiencies and improve patient quality including through medication management. Though inpatient settings may have historically conducted these internal value assessments, post-acute facilities including Skilled Nursing Facilities and Behavioral Health settings are limited in their capacity to adopt new technologies. We recommend that CMS work with industry, hospital, and pharmacy leaders to envision operational and clinical excellence with a reimbursement pathway to incent it.

Accounting for Capital Investment in AI Technologies under Medicare's Payment Systems

Despite the advancements in prevention, treatments and therapies, challenges to access and disparities in care persist in the US health system. Factors such as geographic inaccessibility, economics, insurance, QHP shortages, health system bias, race and ethnicity, language and many other factors contribute to this growing trend. In addition, for many conditions, access to evidence-based first line therapy is limited at best, reducing care quality, negatively impacting population health and increasing total health care costs. The very nature of AI provides opportunities to achieve access to evidence-based care at scale, especially for underserved populations. These innovative technologies have been proven to deliver clinically effective treatment and allow for ongoing care to reach health care deserts where access to QHPs is challenged and where existing treatments fall short.

Modernizing Fee-for-Service Payments to Support AI Adoption

We also encourage CMS to consider holistic updates to its fee for service payment systems to reflect the significant resource investments required to adopt and implement not only AI within medical devices, but the broader scope of workflow and process improvement software and related technologies. These investments extend beyond procurement of a software license and include cybersecurity updates, staff training, and hardware procurement. While these costs are typically considered indirect and therefore not reimbursable, we encourage the agency to consider how they might be incorporated into annual market basket updates or other update factors to ensure appropriate accounting for provider costs.

Finally, aligning regulatory and reimbursement policies will be critical to ensure AI tools integrate into clinical workflows and are evaluated based on real-world performance, reducing uncertainty and accelerating responsible adoption.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

As we all know, innovation moves at the speed of trust – particularly in clinical care. Supporting private-sector innovation by promoting greater clarity, alignment, and stability across AI accreditation, certification, and governance frameworks will be critical in accelerating adoption.

FDA has established a strong, trusted foundation for regulating AI-enabled medical technologies, and continued alignment with FDA - along with international harmonization (e.g., IMDRF) - is critical to maintaining trust, enabling global competitiveness, and ensuring U.S. leadership in the AI race. It's equally important to establish alignment (e.g., standards, best practices, success metrics) with relevant professional societies and hospital accreditation bodies that shape how AI is operationalized in hospitals and other care settings. From an industry perspective, harmonized and consistent AI regulations, standards, and best practices across public and private sectors are critical to accelerating innovation and adoption.

6. What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

The greatest potential for AI to improve healthcare outcomes, enhance quality insights, and reduce costs lies in technologies that operate at the intersection of clinical decision support, workflow automation, and closed-loop care delivery.

- A. **Closed-loop technologies** hold significant promise to advance deflationary, workforce supporting care delivery by safely automating well-defined clinical tasks through tightly governed connections between monitoring, analytics, and therapeutic action. When appropriately designed and overseen, these systems can reduce cognitive and administrative burden on clinicians, extend scarce clinical capacity in rural and underserved settings, and enable more timely, consistent interventions.

For instance, Medication errors remain one of the most persistent and costly patient safety challenges in healthcare. AI tools that integrate pharmacy automation, infusion delivery, and patient monitoring into a unified, closed-loop system have the potential to dramatically reduce adverse drug events, optimize dosing in real time, and ensure the right medication reaches the right patient at the right dose. AI embedded across these connected devices can flag dosing anomalies, reconcile medication orders against patient-specific parameters, and alert clinicians before an error occurs rather than after.

At the same time, we recognize potential concerns around safety, accountability, and oversight of closed-loop systems, underscoring the importance of clear guardrails, staged testing environments, and robust post-market surveillance. We recommend development of a clear, accelerated pathway to responsibly test and reimburse such technologies to improve access, efficiency, and patient outcomes.

- B. **AI-Enabled Hemodynamic and Patient Monitoring:** AI-powered continuous monitoring tools represent a significant opportunity to shift care from reactive to proactive. By analyzing real-time physiological data streams (including blood pressure, cardiac output, fluid responsiveness, and respiratory patterns) AI algorithms can detect early signs of clinical deterioration with a specificity and speed that exceeds traditional threshold-based alerting. Such hemodynamic monitoring technologies, equipped with AI-driven analytics, can help clinicians anticipate and prevent serious complications such as acute kidney injury, hemodynamic instability, and sepsis. In surgical and critical care settings, this translates directly into reduced complications, shorter ICU stays, and lower total cost of care.
- C. **AI-Driven Pharmacy Automation and Intelligent Supply Chain:** Beyond the bedside, AI has significant potential to transform pharmacy operations and medication supply chain management. Intelligent automation tools that leverage AI to optimize medication inventory, predict demand, flag dispensing irregularities, and reduce drug diversion can meaningfully lower operational costs while improving safety and compliance. Pharmacy automation platforms, augmented with AI capabilities, can help health systems move from manual, error-prone processes to predictive, data-driven pharmacy operations therefore resulting in reduced waste, improve throughput, and free pharmacists to focus on higher-value clinical activities.

7. Which role(s), decision maker(s), or governing bodies within health care organizations

have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Establishing clear, cross-functional governance groups within healthcare organizations is essential to accelerating adoption of AI in clinical care. Meaningful decisions typically require alignment among three groups – 1. clinical leaders focused on patient outcomes, 2. finance and procurement teams assessing value and return on investment, and 3. IT and data governance functions responsible for interoperability, cybersecurity, and infrastructure readiness. National guidelines and professional societies further shape adoption by building clinician confidence and standardizing appropriate use of AI.

Our experience through our Clinical Affairs teams working closely with hospitals shows that AI adoption is most successful when technologies are evaluated through multidisciplinary structures. In the absence of coordinated oversight, many AI solutions face “valleys of death” between initial pilots and true scale-up. From best practice standpoint, healthcare systems have addressed similar challenges (e.g., antimicrobial resistance) by establishing dedicated cross-functional committees accountable for outcomes. A comparable governance model for AI, supported by clearer guidance, could help organizations move more efficiently from pilots to meaningful adoption. BD would welcome the opportunity to share our engagement frameworks that have proven effective in demonstrating the clinical, operational, and economic value of AI technologies.

Lastly, health systems face significant administrative hurdles due to reduced funding and tight cost constraints, forcing them to maximize the use of existing capital equipment; without targeted incentives, hospitals have limited ability to move beyond the status quo and adopt new technologies.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability would significantly widen market opportunities and accelerate AI development by reducing the cognitive and operational burden clinicians face from navigating multiple, disconnected systems. Today, clinicians are often required to engage with numerous screens, dashboards, and point solutions, many of which operate outside core clinical workflows, limiting adoption and diminishing the value of AI. Interoperability that enables AI tools to integrate seamlessly with EHRs and other clinical systems - rather than requiring clinicians to leave their primary workflow - would improve usability, trust, and clinical impact. In addition, ASTP should consider creating a national data registry to provide industry access to standardized datasets that support benchmarking and real-world testing.

Enabling this by promoting interoperable data standards (e.g., USCDI-aligned data elements, FHIR-based APIs), supporting consistent identity and patient-matching frameworks, and encouraging benchmarking tools that assess AI performance within real-world clinical workflows. Such an approach would allow a broader ecosystem of innovators to contribute value, streamline how AI engages clinicians, and ensure that competition and innovation are driven by outcomes and usability.

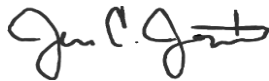
10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Prioritizing applied AI research that addresses high-impact and high-cost public health needs such as maternal health, cardiovascular care, and medication errors is where the initial focus should be in terms of technology adoption. Examples include earlier identification of postpartum hemorrhage, continuous hemodynamic monitoring, and AI-enabled pharmacy automation to support reliable medication availability closer to patients, recognizing that affordability alone does not ensure access. Expanding federal regulatory pilots such as TEMPO to include such critical use cases would support nationally relevant public health priorities while helping establish consistent regulatory expectations across a fragmented payer landscape.

Importantly, AI research should not focus on technology in isolation but also examine the people and process changes required for adoption, including clinician education and certifications, workflow redesign, and appropriate guardrails for safe use. Research that integrates technology with workforce readiness and operational change will be most effective in demonstrating real-world value and accelerating adoption at scale.

BD looks forward to working closely with ASTP to advance our shared priority of accelerating adoption of AI in clinical care and positioning America as a global leader in AI driven medical technologies.

In the meantime, please feel free to contact us with any questions or for additional information.



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